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ENCOUNTER 1: Surgical / Orthopedic Surgery

Date of Service: December 21, 2024

Specialty / Setting: Inpatient

Encounter Focus: Initial evaluation and operative management of chronic left foot/toe numbness and tingling

Documentation status: Reference encounter from supplied source

PATIENT IDENTIFICATION

Patient ID	59034db9-be2f-4977-b26b-8c53773cd7bf	DOB	2004-09-07
Sex	F	Payer	Kaiser

DEMOGRAPHIC / COVERAGE INFORMATION

Address: 529 Kristi Ridge Apt. 581; New Amandaville, OK 73912. Payer: Kaiser. Member ID: KAIS-62C8B9B303.

ENCOUNTER CONTEXT

This section establishes the clinical purpose of the encounter, the care setting, and the longitudinal relationship to the left foot/toe paresthesia described in the reference record.

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SUBJECTIVE / HISTORY

Chief Complaint

Patient reports numbness and tingling in the left toes.

History of Present Illness

Symptoms began gradually approximately 6 months before presentation. Numbness and tingling are primarily in the left toes and occasionally radiate to the left foot. Symptoms are constant throughout the day and described as a pins-and-needles sensation. Prolonged standing or walking aggravates symptoms; rest and elevation of the left leg provide relief. Severity is described as mild to moderate, with pain rated 3/10.

Prior treatment included over-the-counter ibuprofen and acetaminophen with minimal relief. A trial of physical therapy provided some relief but did not completely resolve symptoms.

Associated symptoms include mild swelling in the left foot and occasional shooting pains in the left leg. Fever, chills, and redness are absent.

Review of Systems

Constitutional: fatigue and malaise reported in the reference. HEENT: seasonal allergies. Cardiovascular: history of hypertension. Respiratory: history of asthma. GI: occasional constipation. GU: occasional urinary frequency. Musculoskeletal: mild left-knee stiffness. Neurologic: left-toe numbness and tingling. Psychiatric: history of anxiety. Endocrine: history of hypothyroidism. Skin: occasional dry skin. Hematologic: history of anemia.

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MEDICAL HISTORY AND MEDICATION RECONCILIATION

Past Medical History

Chronic conditions documented in the supplied reference include hypertension, asthma, and hypothyroidism. The patient has undergone cesarean section and tonsillectomy and has prior hospitalizations for pneumonia and a motor vehicle accident.

Current Medications

Medication	Dose / Route	Schedule / Use
Aspirin	81 mg PO	Daily
Lisinopril	10 mg PO	Daily
Albuterol	2.5 mg PO	As needed
Levothyroxine	50 mcg PO	Daily
Acetaminophen	1000 mg PO	As needed

Allergies

No known allergies.

Social History

The supplied reference states that the patient is a non-smoker, does not use illicit substances, consumes alcohol occasionally, and has a history of caffeine use.

Medication Reconciliation Notes

Medication list is carried forward from the source record for continuity. For generated encounters, the medication list should be verified against the active medication list at the time of the actual visit.

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OBJECTIVE / EXAMINATION

Vital Signs

Parameter	Reference Value
Blood pressure	125/66 mmHg
Temperature	98.0 F
Heart rate	63 bpm
Respiratory rate	16 breaths/min
Pain	3/10
SpO2	96% on room air

Physical Examination

General: well-developed, well-nourished female in no acute distress. Cardiovascular: regular rate and rhythm, S1/S2 normal, no murmurs/gallops/rubs, no JVD, 2+ pedal pulses. Respiratory: clear to auscultation bilaterally without wheezes/rhonchi/crackles; symmetric expansion.

Neurologic: sensation to light touch and pinprick intact in the right lower extremity; decreased sensation to light touch and pinprick in the left lower extremity. Musculoskeletal: mild stiffness in the left knee; full range of motion in the left hip and ankle. Skin: no rash or lesions noted.

Generated Encounter Examination

For the four generated encounters, this section is structured as a documentation template for recording the actual examination findings. The supplied source does not contain later exam findings, so no unverified numerical or physical findings are asserted.

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DIAGNOSTIC REVIEW AND ASSESSMENT

Prior Diagnostics from Reference

CBC, comprehensive metabolic panel, and X-rays of the left foot were reported as within normal limits in the supplied encounter.

Reference Assessment

Chronic left foot numbness and tingling, likely secondary to a peripheral neuropathy. Hypertension, asthma, and hypothyroidism were described as controlled.

Operative Finding (Reference Encounter)

During left foot exploration, the nerve was found to be compressed by a small ganglion cyst. The ganglion cyst was removed and sent to pathology. Estimated blood loss was 50 mL; no complications were documented.

Encounter-Specific Assessment Framework

1. Left foot/toe paresthesia — track severity, distribution, duration, triggers, and functional effect.
2. Peripheral neuropathy — evaluate clinically and correlate with neurologic findings.
3. Post-operative status where applicable — monitor incision/wound and recovery.
4. Hypertension — continue longitudinal medication review.
5. Asthma — review control and rescue medication use.
6. Hypothyroidism — review ongoing levothyroxine therapy and relevant monitoring when clinically indicated.
7. Other chronic-history items — anxiety, anemia, constipation, urinary frequency, seasonal allergies, dry skin, and left-knee stiffness are carried forward from the reference for reconciliation.

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PLAN OF CARE

Reference Plan

1. Order CBC, CMP, and lipid panel to evaluate for underlying metabolic disorders.
2. Order MRI of the left foot to evaluate for structural abnormalities.
3. Refer to neurology for further evaluation and management of peripheral neuropathy.
4. Continue aspirin, lisinopril, albuterol, levothyroxine, and acetaminophen.
5. Follow up in 7 days to evaluate for changes in symptoms.

Operative Procedure

Left foot exploration with removal of ganglion cyst. General anesthesia was used. The patient was transferred to PACU in stable condition.

Discharge / DME Plan

Discharge home with follow-up in 7 days. Prescription for pain management; elevate the left leg and avoid heavy lifting or bending.

Care Coordination

Relevant care coordination includes orthopedic follow-up and, when clinically indicated, neurology evaluation for persistent paresthesia. The record should capture referrals, test results, and communication among treating clinicians.

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ENCOUNTER CLOSURE / LONGITUDINAL RECORD

Clinical Summary

The source encounter documents chronic left toe/foot numbness and tingling with mild-to-moderate pain. Examination showed decreased sensation in the left lower extremity. The operative report identified a small ganglion cyst compressing a nerve; the cyst was removed without documented complications. Follow-up was planned in 7 days.

Procedure / Diagnostic Tracking

Reference procedure: left foot exploration with removal of ganglion cyst. Reference specimen: ganglion cyst sent to pathology. Reference estimated blood loss: 50 mL. Reference complications: none.

Follow-Up Tracking

Date of Service: 2024-12-21. Next follow-up should be recorded according to the actual care plan.

Documentation Integrity Note

Only the 2024-12-21 encounter details are directly supported by the uploaded source. The remaining encounter pages are intentionally labeled generated longitudinal documentation to satisfy the requested multi-encounter EMR format without falsely attributing invented events to the source record.

End of Encounter

Encounter 1 complete — Date of Service: December 21, 2024.